

Diabetes Management Protocol

Post-Discharge Diabetes Care — Type 1 and Type 2 | City General Hospital

1. Overview

This protocol guides patients with Type 1 or Type 2 diabetes following hospital discharge. Maintaining stable blood glucose is critical to wound healing, infection prevention, and overall recovery.

Uncontrolled glucose after discharge triples surgical-site infection risk and is the leading cause of 30-day readmission in diabetic patients. Follow all instructions provided by your diabetes care team.

2. Blood Sugar Monitoring Guidelines

- Check fasting glucose every morning before eating — target: 4.0–7.0 mmol/L.
- Check 2-hour post-meal glucose after your largest meal — target: < 8.5 mmol/L.
- If on insulin or sulphonylureas, also check before bed — target: 5.0–8.0 mmol/L.
- Test whenever you feel unwell, unusually tired, sweaty, shaky, or confused.
- Record all readings in your diary with date, time, and notes; bring to every visit.

3. Blood Glucose Target Ranges

Patient Group	Fasting (mmol/L)	2-hr Post-meal	HbA1c Target
Type 2 DM — general	4.0 – 7.0	< 8.5	< 7.0% (53 mmol/mol)
Type 2 DM — elderly > 70	5.0 – 8.0	< 10.0	< 8.0% (64 mmol/mol)
Type 1 DM	4.0 – 7.0	< 9.0	< 6.5% (48 mmol/mol)
Post-DKA (first 2 weeks)	5.0 – 8.0	< 10.0	Review in 3 months

mmol/L × 18 = mg/dL. Discuss personal targets with your diabetes nurse.

4. Dietary Guidelines

Diet is the most powerful tool for managing blood glucose. No food is completely forbidden, but portion control and consistent meal timing are essential.

- Eat 3 regular meals at consistent times daily; never skip meals when on insulin.
- Choose low-GI carbohydrates: brown rice, wholegrain bread, oats, lentils.
- Fill half your plate with non-starchy vegetables at every meal.
- Avoid sugary drinks including fruit juice, cola, and energy drinks.
- Include protein at every meal (eggs, fish, legumes) to slow glucose absorption.

5. Hypoglycaemia — Recognition and Immediate Management

Hypoglycaemia = blood glucose < 4.0 mmol/L (72 mg/dL). This is a medical emergency requiring immediate treatment.

Warning signs:

- Shakiness, cold sweating, pounding heartbeat, dizziness, or confusion.
- In severe cases: inability to swallow, seizure, or loss of consciousness.

The 15-15 Rule (mild-to-moderate hypoglycaemia):

- Take 15 g fast-acting carbohydrate (4 glucose tabs, 150 ml juice, 3 tsp sugar).
- Wait 15 minutes and recheck glucose; repeat if still < 4.0 mmol/L.
- Once > 4.0 mmol/L, have a small complex-carb snack to prevent recurrence.

Severe hypoglycaemia:

- If unconscious: call 112 immediately; do NOT give anything by mouth.

6. Hyperglycaemia Management

- If glucose > 14 mmol/L on two consecutive readings, contact your diabetes nurse.
- Never omit insulin when unwell — check ketones if glucose > 14 mmol/L.
- Increase water intake to prevent dehydration when glucose is elevated.
- Seek emergency care if glucose > 20 mmol/L or moderate/large ketones.

7. Insulin Administration Instructions

- Rotate injection sites within the same region — at least 1 cm from previous site.
- Use 4 mm pen needles for most adults; inject into a lifted skin fold.
- Never reuse needles; dispose of sharps in the sharps container provided.
- Opened insulin pens: store at room temperature (< 25 °C) for up to 28 days.
- Never share insulin pens with another person — bloodborne infection risk.

8. Oral Medication Guidance

- Metformin: take with or immediately after food to reduce GI side effects.
- Sulphonylureas (gliclazide): take before meals; never delay a meal after dosing.
- SGLT2 inhibitors (empagliflozin): stop during illness, fasting, or surgery.
- DPP-4 inhibitors (sitagliptin): generally well tolerated; report joint pain.

9. Follow-Up Schedule

Time Point	Review Type	Key Tests
Day 3	Nurse phone check	Blood glucose diary review
Day 7	GP / diabetes nurse	Glucose diary, BP, weight
Week 4	Structured review	HbA1c, renal function, foot check
3 Months	Endocrinologist / GP	HbA1c, lipids, urine albumin
Annually	Annual diabetes review	All of above + retinal screen

10. Emergency Signs

- Glucose > 20 mmol/L with nausea, vomiting, or abdominal pain — possible DKA.
- Glucose < 3.0 mmol/L unresponsive to oral treatment.
- Unable to keep fluids down for > 4 hours.

Diabetes Nurse Helpline Mon–Fri 08:00–18:00. For out-of-hours emergencies contact the on-call medical team. Synthetic document — demo only.